

MENOPAUSE

- Exclude pregnancy before diagnosing menopause ↗ 157.
- Menopause is no menstruation for at least 12 months in a row. Most women have menopausal symptoms and irregular periods leading up to menopause.
- If menopausal and < 40 years, discuss with specialist.

Assess the menopausal patient

Assess	When to assess	Note
Symptoms	Every visit	• Ask about menopausal symptoms: hot flushes, night sweats, vaginal dryness, mood changes, difficulty sleeping ↗ 87 and sexual problems ↗ 58. • If night sweats, ask about other TB symptoms : if cough, weight loss or fever, exclude TB ↗ 92. • Manage other symptoms as on symptom pages.
Vaginal bleeding	Every visit	If bleeding between periods, after sex or after being period-free for 1 year, refer within 2 weeks.
Depression	Every visit	In the past month, has patient: 1) felt down, depressed, hopeless or 2) felt little interest or pleasure in doing things? If yes to either ↗ 143.
Osteoporosis risk	At diagnosis	Refer for possible treatment if high osteoporosis risk: < 60 years with loss of > 3cm in height or fractures of hip/wrist/spine, previous non-traumatic fractures, oral steroid treatment for > 3 months, onset of menopause < 45 years, BMI < 18.5, heavy alcohol user, heavy smoker.
Family planning	At diagnosis	• If < 50 years, give contraception for 2 years after last period. • If ≥ 50 years, change to progestogen-only or non-hormonal contraceptive until 1 year after last period ↗ 154.
BP	3 monthly on HT ¹	If known hypertension ↗ 133. If not, check BP: if ≥140/90 ↗ 132.
CVD risk	At diagnosis	Assess CVD risk ↗ 127.
Breast check	At diagnosis, 6 monthly	If lump/s found in breasts or axillae, refer same week to breast clinic. If available, arrange mammogram at HT ¹ initiation.
Cervical screen	When needed	If HIV negative: do 3 cervical screens, each 10 years apart from age 30 ↗ 55; if HIV positive: do cervical screen at HIV diagnosis (regardless of age), then 3-yearly ↗ 55.
Thyroid	At diagnosis	If weight change, pulse ≥ 100, tremor, weakness/tiredness, dry skin, constipation or intolerance to cold or heat, check TSH. If abnormal, refer to doctor.

Advise the menopausal patient

- To cope with the flushes, advise patient to dress in layers and to decrease alcohol, avoid spicy foods, hot drinks and warm environments.
- If patient is having mood changes or not coping as well as in the past, refer to counsellor, support group or helpline ↗ 178.
- Educate that long term use of hormone therapy (HT) can increase risk of breast cancer, deep vein thrombosis (DVT) and cardiovascular disease. It can be used to treat menopausal symptoms for up to 5 years.

Health for All

↗ 62

Treat the menopausal patient

- If menopausal symptoms interfere with daily function, treat with hormone therapy (HT) if no contraindications². If dose range given, start with lowest dose and increase until symptoms improve.
 - If patient has had uterus removed (hysterectomy): give only **estradiol** 1-2mg daily or **conjugated estrogens** 0.3mg-1.25mg daily.
 - If patient still has a uterus (no hysterectomy), choose HT according to menstruation pattern:

If ≥ 1 year since last period, give:

- **Conjugated estrogens** 0.3-0.625mg and **medroxyprogesterone** 2.5-5mg daily or
- **Estradiol/norethisterone** 1mg/0.5mg daily or
- **Estradiol/norethisterone** 2mg/1mg daily.

If still menstruating/recently stopped, give:

- **Estradiol/cyproterone** 1 tablet daily (**estradiol valerate** 2mg for 11 days, followed by **estradiol valerate/cyproterone acetate** 2mg/1mg for 10 days, then placebo tablet for 7 days).
- **Estradiol** 1-2mg daily for 21 days with **medroxyprogesterone** 5-10mg daily from day 12-21, followed by no therapy from day 22-28 or
- **Conjugated estrogens** 0.3-0.625mg daily for 21 days with **medroxyprogesterone** 5-10mg daily from day 12-21, followed by no therapy from day 22-28.

- Treat vaginal dryness and pain with sex with lubricants (avoid petroleum jelly with condoms). If no better with HT or if HT contraindicated, refer.
- Review 6 monthly once on HT. Decrease/stop if symptoms are controlled. If ≥ 5 years of HT or patient ≥ 60 years, stop treatment. If still symptomatic, refer to specialist.

¹Hormone therapy. ²Avoid if ≥ 60 years, abnormal vaginal bleeding, cancer of uterus or breast, previous deep vein thrombosis or pulmonary embolism, recent heart attack, liver disease, porphyria (rare hereditary disorder).